Adjuvant abemaciclib + endocrine therapy

Abemaciclib is taken twice daily continuously, with endocrine treatment, for up to two years if appropriate. A 28-day block is shown for orientation; there is no planned tablet-free week. Endocrine treatment usually continues longer.

Draft — independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Diarrhoea / dehydration

Abemaciclib

Often starts early. Begin your prescribed diarrhoea plan and contact the team; persistent symptoms may require a treatment hold.

3 Liver injury

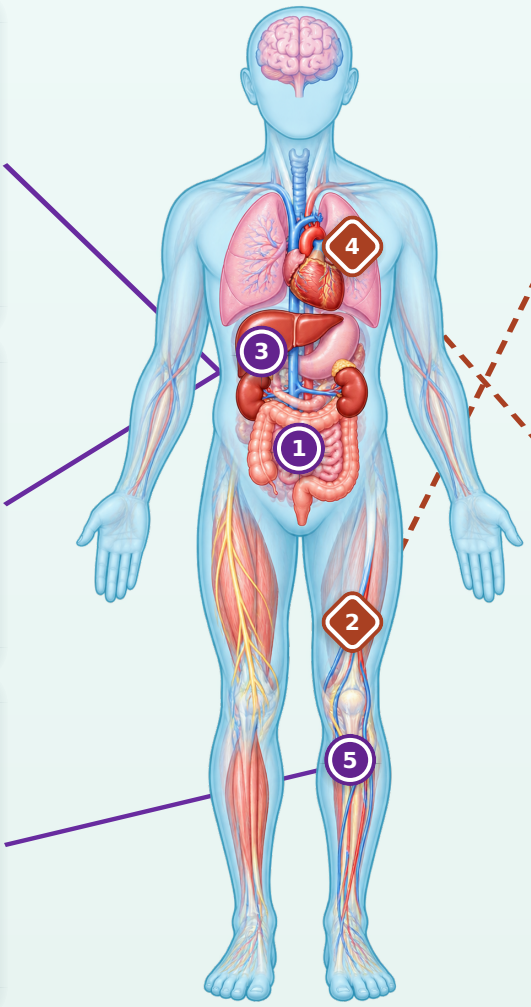
Abemaciclib

Keep liver blood tests. Report jaundice, dark urine or upper-abdominal discomfort promptly.

5 Blood clot

Abemaciclib

A painful swollen leg or new chest / breathing symptoms needs urgent assessment. Severe breathlessness or chest pain: emergency help.



2 Low neutrophils / infection

Abemaciclib

Keep blood tests. Fever, shivers or sudden illness: contact oncology immediately.

4 Lung inflammation

Abemaciclib · uncommon but important

New cough or breathlessness needs prompt assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Abemaciclib

- Diarrhoea, dehydration and nausea
- Low blood counts and infection
- Liver-test abnormalities
- Blood clots
- Rare serious lung inflammation

Your endocrine treatment

- Hot flushes, joint or muscle symptoms
- Vaginal dryness and sexual effects
- Bone loss with AI / ovarian suppression
- Clot and uterine risks with tamoxifen

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Diarrhoea plan

Have the prescribed antidiarrhoeal medicine available. Start the agreed plan at the first loose stool and call early.

Daily tablets

Take the prescribed dose at regular times. Do not double up after a missed or vomited dose.

Baseline and ongoing checks

Baseline FBC, renal, liver and bone profile. Baseline ECG as clinically indicated. Bloods every two weeks for two months, then monthly for two months, then as indicated. No routine ribociclib-style Day 14 ECG schedule.

Endocrine and bone care

Confirm endocrine/OFS treatment, bone-health assessment and the longer-term follow-up plan.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

CUH · MON–FRI 8AM–5PM

087 181 9645

Acute Oncology Service

CUH · WEEKENDS / BANK HOLIDAYS 9AM–5PM

021 4922000

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00619/

Sources and review status are available online. Follow your own prescription and hospital instructions.